

Document Reference Number UHN-PO-HR07

Policy/Guideline Title:	On-Call Policy		
Executive Summary:	University Hospitals of Northamptonshire Group is committed to operating a fair and consistent framework in which employees carry out on-call arrangements and receive appropriate pay enhancements to reflect the out-of-hours services being provided across the Group.		
Supersedes:	UHN On-Call Policy v1		
Description of Amendment(s):	Reformatted into current template and compensatory rest section updated		
This policy will impact on:			
Policy Area:	People Division	Approval Date:	November 2024
Version Number:	1.1	Review Date:	November 2025
Issued By:	Paula Kirkpatrick, Chief People Officer	Expiry Date:	28 April 2026
Author:	Sarah Kinsella, HR Business Partner – Governance and Compliance	Impact Assessment Date:	14 November 2022
APPROVAL RECORD			
	Committees / Group	Date	
Consultation:	n/a minor amendments	n/a	
Endorsed by Head of Function:	Paula Kirkpatrick, Chief People Officer	20/11/24	
Ratified by:	UHN Policy Ratification Group	Chair Approval November 2024	
Received for Information:			

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Dedicated to Excellence Values	What our values mean	How this applies to the policy
 ► Compassion	We care about our patients and each other. We consistently show kindness and empathy and take the time to imagine ourselves in other people's shoes.	• Our On-Call Policy should be applied with compassion genuinely caring for the well-being of our colleagues. • Managers consistently show kindness and empathy, taking the time to understand colleagues challenges, needs and personal circumstances
 ► Accountability	We take responsibility for our decisions, our actions and our behaviours. We do what we say we will do, when we say we will do it. We acknowledge our mistakes and we learn.	• Colleagues and managers take responsibility for their decisions, actions, and behaviours, ensuring that they fulfil their commitments and meet expectations in relation to their on-call duties hours.
 ► Respect	We value each other, embrace diversity and make sure everyone feels included. We take the time to listen to, appreciate and understand the thoughts, beliefs and feelings of others.	• Colleagues and managers are asked to respect the measures put into place to ensure we are safeguarding the overall health and safety, welfare of our workforce and fostering a healthy and respectful workplace for all our colleagues and our patients.
 ► Integrity	We are consistently open, honest and trustworthy. We can be relied upon, we stand by our values and we always strive to do the right thing.	• We consistently demonstrate openness, honesty, and trustworthiness in all aspects of relationships at work.
 ► Courage	We dare to take on difficult challenges and try out new things. We find the strength to speak up when it matters and we see potential failure as an opportunity to learn and improve.	• We find the strength to speak up when it matters, constructively sharing our opinions and concerns about on-call duties and working hours.

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1. INTRODUCTION

On-call systems are part of arrangements to provide appropriate out-of-hours service cover across the NHS and staff on-call are entitled to receive an on-call payment.

A member of staff is on-call when, as part of an established arrangement with University Hospitals of Northamptonshire Group (UHN), when they are available outside their normal working hours – either at the workplace, at home or elsewhere – to work as and when required.

This policy details the framework within which on-call arrangements will be agreed in line with the Agenda for Change (Annex 29 NHS Terms and Conditions of Service Handbook) principles for harmonised on-call payments and other extended service cover.

This policy may be implemented and become applicable when:

- Services review their on-call provision
- Implementation of new on-call arrangements
- Group structural reviews take place

The expectation is that there will be a gradual review of on-call provisions across the UHN group with existing arrangements remaining in place.

2. PURPOSE

The purpose of the policy is to set out a clear, consistent and fair framework for on-call arrangements and payments across the UHN Group.

In line with the principles of equal pay for work of equal value, this policy has been drawn up to ensure that staff required to be available for work out of normal hours receive fair and equal payment, both in terms of them having to be available and for actual work undertaken when called.

In operating this policy, the Trust aims to:

- ensure equal pay for equal work
- ensure the framework for on-call / standby is consistent within the UHN Group and is embedded across the Group
- acknowledge that different departments and services have different make up and needs
- determine the arrangements for the future of on-call working
- ensure that pay enhancements for on-call are reflective of pay increases, annually

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3. SCOPE

This policy applies to all staff required to provide on-call cover and who are employed on Agenda for Change Terms and Conditions.

This policy does not apply to staff who fall outside of Agenda for Change Terms and Conditions.

It is acknowledged that there is not a 'one size fits all' approach to on-call, therefore the policy provides a framework to review current on-call arrangements and ensure that they are aligned to the principles as per Annex 29 of the Agenda for Change Terms and Conditions.

4. SUBSTANTIVE CONTENT

4.1. On-Call Principles

Principles for harmonised on-call arrangements as per Annex 29 of Agenda for Change Terms and Conditions are as follows:

Issue	Principles
1. Equal pay	The guiding principle should be that the harmonised arrangements should be consistent with the principles of equal pay for work of equal value. The effect of this should be that schemes agreed should provide consistent payments to staff at the same pay band available at the same on-call frequency. All departments will need to undertake an equality assessment of their proposals.
2. Commitment or availability	There needs to be a payment to reflect the availability for being called. There are three distinct types of on-call availability: 1. At home ready to be called out or to undertake work at the workplace. 2. At work ready to undertake work. 3. Sleeping in at a workplace. Payment for these different types of availability – options include: - Flat rate available for all staff - Flat rate by band - Percentage of salary. This payment will reflect the frequency of commitment. If the department decides to use a flat rate, they will need to agree arrangements for uprating this payment when pay increases. In setting the availability payment, departments will need to take account of the commitment to work weekends and public holidays.

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	Where tiered on-call systems are required, there should be no distinction between levels of commitment when setting the availability / commitment payment.
3. Frequency	That part of the week covered by on-call arrangements should be divided up into appropriate periods for the purposes of calculating the frequency of on-call availability.
4. Work done	Payment for work done, including work done at home, should be made at the appropriate hourly rate with reference to the Agenda for Change Terms and Conditions Handbook. Departments may agree an appropriate minimum payment period for work done.
5. Time off in Lieu (TOIL)	Staff should have the option to take TOIL rather than payment for work done.
6. Compensatory rest	Individuals will receive compensatory rest for work done, in accordance with section 27 of Agenda for Change Terms and Conditions Handbook.
7. Travel to work	As per current arrangements. Travel time should be paid at the rate agreed for on-call work done, and departments will need to identify if there is a minimum and/or maximum time claim identified.
8. Public holidays (PH)	Covering a PH will attract a day in lieu in accordance with paragraph 13.4 of the Agenda for Change Terms and Conditions Handbook, irrespective of work done. Work done on public holidays would attract payment at the appropriate rates as identified in paragraph 13.4.
9. Sleeping in	A sleeping-in session will often incorporate the following elements: - Hours of wakefulness - Sleep - Work done The term 'sleeping-in' does not refer to individuals who are on-call from the workplace and are able to sleep between periods of work. Under the working time regulations if an individual is required to sleep in at a workplace this counts as working time. However, time asleep does not count for the purposes of the minimum wage. If asleep, this working time does not count for the purposes of the minimum wage. Under the Minimum Wage Regulations, the availability payment should be at least the same as a calculation for (hours of expected wakefulness minimum wage). Local partnerships will need to consider if it is more appropriate to base this calculation

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	on the bottom point of the Agenda for Change pay scales, as described within the Handbook. In those situations where a sleeping-in session includes what the National Minimum Wage Regulations would classify as work, or when the individual is woken during a sleeping-in duty, this should be paid as work done at the appropriate hourly rate. Departments may agree a minimum payment period for work done.
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4.2. Payments for On-Call

Payments will be made based on:

- availability for being called
- actual work undertaken when called.

4.3. Pay Associated with Availability for being On-Call

If you are required to be available to provide on-call cover outside your normal working hours, you will be entitled to receive a pay enhancement.

This enhancement recognises both your availability to provide cover and any advice given by telephone during periods of on-call availability.

The enhancement paid will be dependent upon the proportion of on-call periods in the rota when on-call cover is provided.

The on-call period in each week should be divided into nine periods of at least 12 hours. The enhancement for you will be based on the proportion of these periods in which you are required to be on-call.

Each department requiring an on-call service will determine locally the on-call period required to provide out-of-hours cover for the service.

Table 1 – Agenda for Change example of Pay Enhancement Percentages per Frequency of On-Call

Frequency of On-Call	Value of Enhancements as a Percentage of Basic Pay
1 in 3 or more frequent	9.5 per cent
1 in 6 or more but less than 1 in 3	4.5 per cent
1 in 9 or more but less than 1 in 6	3 per cent
1 in 12 or more but less than 1 in 9	2 per cent
Less frequent than 1 in 12	1 per cent

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4.4. On-Call Payments for Part-Time Staff or other Staff working non-standard hours

If you work part time or work hours other than 37.5 hours per week excluding breaks, the percentage added to your basic pay relating to on-call availability will be adjusted to ensure you are paid a fair percentage enhancement of salary for on-call working.

This will be done by adjusting the payment in proportion to your part time salary so that you receive the same payment for the same length of availability on-call as full-time staff.

4.5. Pay Associated with Work Undertaken when Called

If you are called into work during a period of on-call you will receive payment for the period you are required to attend, including any travel time. Alternatively, you may choose to take time off in lieu. However, if for operational reasons time off in lieu cannot be taken within three months, the hours must be paid for.

For work (including travel time) as a result of being called out, you will receive a payment at time and a half, with the exception of work on general public holidays which will be at double time. Time off in lieu should be at plain time.

There is no disqualification from this payment for bands 8 and 9, as a result of being called out.

There will be a minimum period of two hours' payment for work undertaken onsite, including travel time. Where second requests are received within the initial two-hour period, both calls will be considered as one period of work (i.e., there will not be a further minimum payment of two hours).

The period used to calculate payment will start from the receipt of the phone call and will end when the employee arrives home. There will be a minimum period of one hours' payment for work undertaken off site (i.e., working remotely from home on the computer).

4.6. Other arrangements to provide extended service cover (Stand-By Cover)

You may be required to be on the premises to provide emergency cover but are allowed to rest, except for the times when you are required to carry out emergency work.

If this is necessary, the same payment arrangements will be in place as with the department's agreed on-call arrangements.

4.7. On-Call Over a Public Holiday

If you are required to be on-call on a public holiday you will receive your availability payment and any payment for work undertaken, plus TOIL equivalent to the day you would have worked on the public holiday (e.g. this would be a 7.5 hour day not a whole on-call period of 12 or 24 hours).

4.8. General Provisions

No on-call payments will be received if you are absent from work.

Ideally, you should not be rostered on to an on-call shift that would go into a day off. However, if you are asked to cover due to unforeseen circumstances (e.g., sickness), you should receive TOIL to the nearest half day.

If you volunteer to be on-call on a day off, you are agreeing to work overtime and should be paid for this instead of having the day off.

4.9. Payment for telephone calls

A minimum of 15 minutes is assumed for payment of work undertaken by telephone. Where a second telephone call is received within the initial 15 minute period, both calls are considered as one period of work.

Arrangements for contacting staff on-call, which may include UHN mobile phones, private home phones, private mobile phones and pagers, will be reviewed by a Senior Manager to minimise cost, but maintain convenience and reliability. A UHN mobile phone or pager will be provided as needed.

Itemised telephone bills, which have business calls highlighted, will be reimbursed at the actual cost of the telephone calls plus VAT. Where a charge is made for an itemised bill, this will also be reimbursed.

Telephone line rental will not be reimbursed to you, but alternative methods of contact may be provided.

4.10. Travel

Mileage:

When you are required to return to or to attend work outside your normal hours of duty as part of an on-call arrangement, you will be entitled to claim travel expenses for the return journey between home and work.

Travel expenses will be reimbursed at business mileage rates or lease car rates as appropriate.

Travel time:

When you are required to return to or to attend work outside your normal hours of duty as part of an on-call arrangement, travel time will be paid for work undertaken from the time you leave your home until you arrive at your work location and from the work location to home, up to a maximum of one hour each way, subject to service needs.

If you do not attend work straight after the phone call and a specific time to attend is arranged, this will only be paid from the time you leave home.

Where you remain at work after your on-call and do not go home, the return journey will not be paid.

Alternatively, you may choose to take TOIL (at plain time rates) for the actual time spent travelling, where this can be accommodated by the service (and without backfill).

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Departments will clarify the travelling distance or time period within which on-call staff should be able to get to site and provide the rationale for this.

4.11. Compensatory rest

We recognise that there will be some occasions when the full entitlements to daily, weekly and at work rest periods cannot practically be achieved, for instance:

- Where a colleague changes shifts (e.g., from a late shift to early shift)
- Where a colleague works split shifts (e.g., a morning shift and an evening shift)
- Where there is the need for continuity of care in areas where colleagues work in direct contact with patients in “round the clock” services.
- Where there is the need for continuity of a service to support colleagues who work in direct contact with patients in “round the clock” services.
- Where colleagues have undertaken work during an on-call period which is preceded and/or followed by a period of duty.
- Where colleagues are required to work due to emergency or unforeseeable circumstances.

If you have not been able to achieve your rest period, you will be able to take an equivalent period rest, known as compensatory rest.

Compensatory rest is unpaid rest, usually taken in future rest period to ‘replace’ the time of your unpaid break (or part of it) that has been missed or broken. All compensatory rest time will be given like for like i.e. it equals the same amount of time as the break, or part of it, that has not been taken. It should be taken as soon as possible, ideally in the next available rest period, or one soon after, in order for it to be effective and ensure that you are getting sufficient breaks from work. All compensatory rest must be taken within 3 weeks of when the break was missed.

The need for compensatory rest should be kept to a minimum through effective duty rostering/job planning, that ensures that there is provision within all rotas for the correct daily, weekly and breaks at work built in. This is especially important if you undertake on-call or stand-by duties, as this will allow you, where possible, to take rest immediately following an on-call or stand-by period, particularly if it is likely you will be contacted, required to work or have your sleep disturbed.

For more information for please refer to the UHN Working Time Regulations and Secondary Employment Policy, which is available on The Street and KNet.

4.12. Pensions

Availability payments are pensionable, but payment for work done on-call (including travelling time) is not. This is determined by NHS Pensions instructions.

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5. ROLES & RESPONSIBILITIES

ROLE	RESPONSIBILITY
Chief Executive and the Trust Board	Are responsible for ensuring there is a policy in place.
Managers	Are responsible for ensuring that: • Job descriptions detail the contractual requirement to provide on-call cover • The extent of on-call commitment is explained at interview • They agree an on-call rota that complies with the Working Time Directive and that you are aware of your obligations • Systems are in place for the authorisation and checking of on-call payments • They specify who is expected to contribute to the on-call rota and that they realise their commitment to frequency of duty and extent of responsibilities • They ensure that affected employees have been notified in advance and are aware of their commitment • They ensure employees working on-call receive appropriate compensatory rest • They set the time frames for attendance on site, based on clinical need and impact on service, such as delaying discharge Local arrangements are in place for on-call back-up in case of short notice requirements. Reasons for not being available for on-call, such as parental or caring responsibilities or ill health, are discussed and exclusion agreed where appropriate. The manager will seek advice from the People Partnering Team before reaching agreement and guidance may be sought from Occupational Health if required.
Payroll Department	Will ensure: • That on-call payments are made in line with any on-call arrangements agreed in line with this policy • It responds to queries relating to on-call payments, after the employee has discussed any concerns with their line manager in the first instance.

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ROLE	RESPONSIBILITY
All Trust Employees	Participating in on-call arrangements should ensure: • That the hospital is aware of how they can be contacted, and they have provided up-to-date contact details • They are contactable and available for their period of on-call duty, and they attend/respond within any specified time frames • They accurately record time spent working in the appropriate way in a timely fashion and forward it to their manager for submission to payroll • They inform their manager (or nominated deputy) if they are unavailable to be on-call through sickness or emergency as soon as practically possible • They provide an appropriate handover. Have a responsibility to: • Support the Trust to achieve its Vision • Act at all times in accordance with the Trust values • Follow duties and expectations of staff as detailed in the NHS Constitution – Staff Responsibilities

6. DEFINITIONS

Normal Working Hours	Those regularly worked and/or fixed by a contract of employment. Time worked as overtime is not normal work unless an employee's contract specifies a minimum number of overtime hours to be worked. On-call does not apply to staff required to work additional hours immediately after their normal working day as this would be considered overtime. If you are rostered to be on-call and are subsequently required to remain at work as part of your on-call duties, this would attract the appropriate on-call payment.
On-Call	"On-call" is defined as a member of staff being designated as a specific point of contact, outside their normal working hours, and when they are not required to be at work and on site already, so they are available to deal with queries and service-related issues as they arise during the on-call period and/or undertake essential clinical work.
Stand-by	"Stand-by" may be required to be on the premises to provide emergency cover but are allowed to rest, except for the times when you are required to carry out emergency work.

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7. COMPLIANCE STATEMENTS

Equality & Diversity

This document has been designed to promote equality, diversity, inclusion and human rights in line with the Trust's Equality, Diversity and Inclusion Strategies. It has also been analysed to ensure that as part of the Public Sector Equality Duty the Trust has demonstrated that it has given 'due regard' to its equality duty and that, as far as is practicable, this document is free from having a potential discriminatory or adverse/negative impact on people or groups of people who have relevant protected characteristics, as defined in the Equality Act of 2010.

The completed 'equality analysis' assessment tool must be submitted with the policy. The assessment tool must be completed by someone trained to carry out impact assessments.

NHS Constitution

The contents of this document incorporates the NHS Constitution and sets out the rights, to which, where applicable, patients, public and staff are entitled, and pledges which the NHS is committed to achieve, together with the responsibilities which, where applicable, public, patients and staff owe to one another. The foundation of this document is based on the Principles and Values of the NHS along with the Vision and Values of Northampton General Hospital NHS Trust.

Person Identifiable/Confidential Data and Privacy Rights

In line with the UK General Data Protection Regulation (2016) and the Data Protection Act (2018) the Trust is obliged to treat all information in a secure, professional and ethical manner, whilst keeping all person identifiable and personal data confidential. In addition, the Trust will not share employee information with a third party, unless there is a legal basis for disclosure, for example for the detection and prevention of crime, or if it is in the legitimate interest of the Trust.

As part of the Data Security and Protection policies of the Trust and data protection legislation, if the Trust is required to share any reports / information / data relating to the processes and procedures of any of our policies, the data, where possible, will be anonymised to remove person identifiable / confidential data unless there is a justifiable reason not to. It is important that policy leads are aware that policies may be released in response to FOI requests.

For further information regarding a Data Protection Impact Assessment and Sharing Personal Data, please contact the Data Security and Protection Team @ ngh-tr.data.protectionact@nhs.net OR datix.enquiries@nhs.net

8. IMPLEMENTATION & TRAINING

Training relating to this policy will be available from the People Partnering team, who will cascade the policy to relevant managers and ensure supporting information is available on the Trust intranet sites.

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9. MONITORING & REVIEW

Requirement of the policy and compliance to be monitored	Frequency of audit	Responsibility for audit	Results of monitoring reported at:
Audit of Working Time Directive Regulations for those required to undertake on-call	Annually	Head of People Process	ILT

10. REFERENCES & ASSOCIATED DOCUMENTATION

Department of Health (2015). *NHS Constitution: the NHS belongs to us all*. [online]. London. Department of Health. Available from https://assets.publishing.service.gov.uk/government/uploads/system/uploads/attachment_data/file/480482/NHS_Constitution_WEB.pdf [Accessed December 2019]

NHS Terms and Conditions of Service Handbook (2022)

APPENDICES

Appendix 1 Equality Impact Assessment (EqIA)

Equality Impact Assessment (EIA) – Initial Assessment			
1.Division	People Services	2. Department	People Services
3. Person(s) completing this form	Michelle O’Sullivan, Senior HR Advisor	4. Contact Information	Michelle.osullivan5@nhs.net
5. Others involved		6. Start date of this assessment	14 November 2022
7. What is being assessed (please tick)	New Policy ☒ Existing Policy ☐ New Service / Function ☐ Review Service / Function ☐	8. Implementation/ effective date	End of November 2022
9. Name of Policy	On-Call Policy		
10. What are the aims / objectives of policy this service?	This policy details the framework within which on-call arrangements will be agreed in line with the Agenda for Change (Annex 29 NHS Terms and Conditions of Service Handbook) principles for harmonised on-call payments and other extended service cover.		
11. Who will be impacted by policy / service (please tick)	Patients ☐ Carers ☐ Public ☐ Staff ☒ Other ☐	11a. If staff are impacted, how many individuals / Which Groups of Staff are likely to be affected?	All staff required to provide on-call cover.
12. Who has been involved in the policy / service development (please tick)	Patients ☐ Carers ☐ Public ☐ Staff ☒ Other ☒	12a. If yes, who have you involved and how have they been involved	Unions, Managers, HR Team, Solicitors

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13. What further consultation method(s), if any, are you proposing?	Formal consultation through Trust established Trade Union negotiation forum	14. How are any changes / amendments to the policy / service to be communicated?	Formal consultation period
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Equality Impact Assessment – Initial Assessment Outcome

15. As part of the completing the above have you or a member of the group conducting EIA identified any issues relating to the specific service/policy and gender, race, age, religion, physical disability, hearing impairment / sensory impairment / mental illness / learning disabilities / sexual orientation / gender reassignment / marriage & civil partnership / pregnancy & maternity and carer status?	No	☒	EIA Complete (please send completed EIA to kgh-tr.diversity@nhs.net)
	Yes	☐	Requires EIA Full Assessment to be completed.

Equality Impact Assessment – Full Assessment

Review of any previous EIA actions

What actions did you plan last time? (List them from the previous EIA)	What improved as a result? What outcomes have these actions achieved?	What <u>further</u> actions do you need to take? (add these to the Action plan below)
Not applicable	Not applicable	Not applicable

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Review of information, equality analysis and potential actions

Protected characteristics groups from the Equality Act 2010	What do you know? Summary of data about your service-users and/or staff	What do people tell you? Summary of service-user and/or staff feedback	What does this mean? Positive / Neutral / Negative Impact identified from data and feedback (actual and potential)	What can you do? All potential actions to: • advance equality of opportunity, • eliminate discrimination, and • foster good relations
Age	N/A	N/A	N/A	N/A
Disability	N/A	N/A	N/A	N/A
Gender reassignment	N/A	N/A	N/A	N/A
Pregnancy and maternity	N/A	N/A	N/A	N/A
Race/ethnicity Including migrants, refugees and asylum seekers	N/A	N/A	N/A	N/A
Religion or belief	N/A	N/A	N/A	N/A
Sex/Gender	N/A	N/A	N/A	N/A
Sexual orientation	N/A	N/A	N/A	N/A
Marriage and civil partnership	N/A	N/A	N/A	N/A
Community Cohesion	N/A	N/A	N/A	N/A
Other relevant groups	N/A	N/A	N/A	N/A

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Protected characteristics groups from the Equality Act 2010	What do you know? Summary of data about your service-users and/or staff	What do people tell you? Summary of service-user and/or staff feedback	What does this mean? Positive / Neutral / Negative Impact identified from data and feedback (actual and potential)	What can you do? All potential actions to: • advance equality of opportunity, • eliminate discrimination, and • foster good relations
Cumulative impact	All staff required to provide on-call cover will be directly affected by this policy irrespective of protected characteristic	Protected characteristics may be affected by being required to provide on-call cover	Negative	Annual Audit of Working Time Directive Regulations for those required to undertake on call. Review by protected characteristic to establish if there are any negative impacts to be mitigated.

Equality Impact Assessment – Full Assessment Outcome

Assessment of overall impacts and any further recommendations	TBC
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Equality Impact Assessment – Detailed list of data/community feedback that informed your EIA

Title (of data, research or engagement)	Date	Gaps in data	Actions to fill these gaps: who else do you need to engage with? (add these to the Action Plan below, with a timeframe)

Equality Impact Assessment – Prioritised Action Plan¹

Impact identified and group(s) affected	Action planned	Expected outcome	Measure of success	Timeframe

¹ These actions must now be transferred to service or business plans and monitored to ensure they achieve the outcomes identified.

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Equality Impact Assessment – Initial Assessment Outcome

As part of the completing the above have you or a addressed all issues identified as part of EIA initial assessment	No	☐	Please speak to Diversity and Inclusion Manager
	Yes	☐	EIA Complete (please send completed EIA to kgh-tr.diversity@nhs.net)

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